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June 16, 2025

Centers for Medicare & Medicaid Services
P.O. Box 8013
Baltimore, MD 21244-8013

Re: CMS-0042-NC.

Dr. Mehmet Oz:

Thank you for the opportunity to respond to the Request for Information (RFI) on the Health Technology Ecosystem and contribute to this important dialogue by sharing insights most relevant to Contexture and other health information exchange (HIE) organizations.

Contexture is one of the nation's leading HIEs, serving Arizona and Colorado, and the largest health information organization in the Western region. As a nonprofit, we support secure, interoperable data exchange for approximately twenty-eight (28) million patients across thousands of healthcare organizations (*See Figure 1*), including hospitals, physician practices, behavioral health providers, long-term care facilities, and public health agencies.

Founded on the principle that connected care improves outcomes, Contexture operates as a neutral intermediary and infrastructure partner that bridges the gap between disparate healthcare systems, enabling access to longitudinal patient data at the point of care. Our success (*See Figure 2*) reflects sustained public and private investment in high-performing HIEs that are responsive to community needs.

In our response we focus on three areas where CMS can drive impact: (1) accelerating the use of FHIR APIs while recognizing current adoption patterns; (2) leveraging high-performing HIEs as proven, community-based infrastructure; and (3) aligning TEFCA with state-level and domain-specific needs. We offer these insights with the goal of supporting shared objectives.

We welcome continued collaboration with CMS and ASTP/ONC.

Sincerely,

Melissa A. Kotrys, MPH
Chief Executive Officer

I. CMS and ASTP/ONC requested feedback on a Nationwide Provider Directory of FHIR Endpoints, the role of HIEs in supporting patient access to health information, and the evolving role of HIEs within broader health data exchange frameworks.

A. High-Performing HIEs: A Foundational Component of Modern Healthcare Systems *PC-5, PC-10, PC-11, PC-12, PR-1, PR-2, PR-8, TD-2, TD-12, TD-17, VB-2, VB-3, VB-4.

High-performing HIEs like Contexture are essential to today's healthcare ecosystem and already deliver outcomes aligned with CMS priorities. As a trusted, statewide health data utility (HDU), Contexture integrates diverse data sources to provide secure, scalable, and actionable data services for providers, payers, Medicaid, public health agencies, and social services. By connecting fragmented EHR systems and care settings, we offer timely, comprehensive patient records—critical for informed clinical decisions and smooth care transitions.

What sets Contexture and other top-tier HIEs apart is their scale, stability, and trust—rooted in rigorous governance, strong community relationships, and alignment with diverse stakeholders.

HIEs offer measurable cost savings and complement, not compete with, emerging national exchange frameworks. Return on investment (ROI) analyses show that providers using Contexture's proven PatientCare 360 and Results Delivery tools reduced emergency department visits by 29% and inpatient admissions by 40% among Medicaid patients. ACO Partners, a large accountable care organization, saved $2.1 million over two years by using Contexture's HIE services for 58,000. We encourage further research into similar outcomes among other high-performing HIEs.

HIEs are indispensable for regional and state-level data aggregation, especially in public health and cross-network care coordination. Our local roots and understanding of community-specific needs allow us to deliver relevant data that national networks may overlook. Over two decades, Contexture has shown how public investment can build sustainable, high-impact networks. In Colorado and Arizona, we partnered with state agencies to align with policy goals, improving care for Medicaid populations and reducing duplication.

As critical infrastructure, HIEs provide real-time emergency department and inpatient alerts, longitudinal records, and daily-use tools that support care for dual eligibles, high-risk patients, and those with complex chronic conditions. Providers consistently report better care coordination and efficiency, with one home care organization noting how HIE notifications eliminated wasted visits when HIE alerts showed recent patient hospitalization, while another provider halved admissions hospital admissions for its highest utilizers (*See Figure 3*).

Contexture also improves government efficiency. Public health and Medicaid agencies report that partnering with us saves weeks—or months—otherwise spent gathering hospital data manually.

Emerging national models for data exchange should be designed to complement, not displace, these well-established, regionally rooted HIE organizations. CMS should recognize, and build upon, the existing HIE ecosystem, as these organizations serve as a natural complement to federal

frameworks and are uniquely positioned to help CMS achieve digital health transformation goals in a cost-effective and state-aligned manner.

We suggest CMS and ASTP/ONC consider:

- Establishing federal funding or incentive programs to support HIE infrastructure.
- Recognizing and expanding existing HIE infrastructure.
- Integrating HIE data into public health systems, supporting value-added services like population health analytics, care coordination, and quality reporting.

B. National Provider Directory Infrastructure: Architecture and Network Integration *PR-5, PR-10, PA-4, TD-5, VB-15.

A nationwide provider directory of Fast Healthcare Interoperability Resources (FHIR) endpoints could significantly improve access to health information by streamlining how organizations connect. Rather than a single, centralized resource, we recommend a federated model that builds on regional HIEs' infrastructure, expertise, and relationships.

Contexture already maintains up-to-date connections with local providers, including an inventory of the EHR endpoints used by our data senders and the types of data each endpoint supports. Access to longitudinal data is essential for targeting resources and designing value-based interventions. By establishing a national directory using state or regional infrastructure, we can minimize redundant efforts, accelerate technical onboarding, and enable more efficient and reliable data exchange across the country which will support a more strategic and effective resource allocation.

Basic provider contact, and endpoint information should be made freely accessible to ensure broad participation and equitable access. Value-added features such as real-time endpoint status, FHIR capability statements and data availability indicators could be offered as premium, fee-based enhancements. This approach would help fund ongoing directory maintenance, endpoint testing, and quality assurance, while ensuring sustainability and reducing financial pressure on smaller providers. HIEs are well-positioned to deliver these advanced services, including facilitating connections, monitoring endpoint performance, and supporting interoperability.

Contexture also recommends a shared governance structure for the national directory with ASTP/ONC providing policy oversight, HIE networks managing daily operations, and national associations contributing their clinical expertise and guidance. This distributed approach would avoid vendor lock-in and leverage the trusted infrastructure and relationships already in place, ensuring the directory remains secure, and responsive to the needs of the healthcare community.

We suggest CMS and ASTP/ONC consider:

- Establishing a federated national provider directory model that builds upon existing trusted regional networks to minimize redundant efforts and accelerate implementation.
- Advancing point-of-care data access nationwide by investing in a federated national provider director that builds on existing state and regional HIE infrastructure.
- Establishing funding mechanisms to sustain essential directory functions, through a combination of federal support and fee-based services, while enabling advanced features to be offered through cost-recovery or subscription-based models.

II. CMS and ASTP/ONC requested feedback on which data sets would be most valuable if made available through CMS APIs, what barriers currently hinder access to these data sources, and what steps CMS can take to encourage developers to create digital health solutions and integration.

A. FHIR Adoption for Interoperability *PR-5, PR-8, PR-10, TD-14, TD-15, VB-15.

The healthcare industry faces a significant disconnect between the promise of FHIR APIs as the future standard for health data exchange and their limited real-world adoption, particularly in complex multi-stakeholder environments like those in Colorado and Arizona. Contexture, and other high-performing HIEs, acknowledge that healthcare data contributors—including hospitals, clinics, and labs—have not prioritized FHIR-based exchange systems. Instead, these healthcare providers continue to rely on established, widely adopted standards such as HL7 v2, CCD, and IHE profiles, which have proven to be the backbone of scalable and timely data exchange across various care settings.

This pattern of limited FHIR adoption is particularly pronounced among behavioral health, long term care, and home and community-based services, many of whom were excluded from federal incentives under the HITECH Act. Despite slow adoption of FHIR, high-performing HIEs, such as Contexture, are already delivering many of the capabilities that FHIR envisions, often with broader reach, greater usability, and more operational flexibility. These existing systems provide longitudinal patient records aggregated from multiple providers with near real-time updates that support point-of-care decision-making, along with event-based services such as Admission, Discharge, and Transfer (ADT) notifications delivered in real time or batched to fit specific clinical workflows for proactive care coordination.

To advance national interoperability goals, CMS should recognize the critical role that high-performing HIEs play in complementing the adoption of FHIR. Federal policy should support a layered approach that leverages the strengths of both: encouraging FHIR adoption where it adds value, while investing in the mature, standards-based systems HIEs operate today. This strategy ensures that all providers, regardless of technical maturity, can meaningfully participate in data exchange.

B. Patient Access APIs *PR-5.

From Contexture's experience, several categories of APIs hold significant promise for driving innovation and improving support for both patients and providers by making health data more accessible and actionable. Standardized *Patient-Generated Health Data (PGHD) APIs*, including those from wearables and remote monitoring devices, would provide a more comprehensive view of patient health. Additionally, expanding and standardizing *Public Health Reporting APIs* would simplify provider compliance, and the inclusion of *Clinical Decision Support*, *Patient Matching/Identity Resolution*, and *Consent Management APIs* would further empower patients and providers, improve data linkage, and enhance patient control over data sharing.

C. Data Access and Exchange *PR-8, PR-9, PR-10, PC-14, TD-3, VB-14.

Access to real-time or near real-time eligibility and encounter data is critical for effective care coordination and reducing administrative burden. However, HIEs like Contexture cannot provide

these insights unless health plans, including Medicaid and Medicare Advantage plans, are actively contributing data.

With consistent access to eligibility, encounter, and authorization data from payers, regional HIEs offer powerful tools supporting care navigators and reduce information gaps at the point of care. CMS can encourage health plans to share this data through state and regional HIEs. Doing so would strengthen local interoperability, reduce duplicative infrastructure investments, and deliver timely, actionable insights to providers and care teams working across diverse clinical and community settings.

Equally important is the need for a federal policy that prioritizes the adoption of modern identity solutions for authenticating and authorizing access to patient data. Strong identity management is essential to ensure that only authorized individuals can access sensitive health data at the point of care. Solutions, such as multi-factor authentication and dynamic authorization frameworks are crucial for enabling care navigators to securely access real-time eligibility status, prior authorization decisions, and encounter histories in a HIPAA and state privacy law compliant manner. By mandating standardized, interoperable identity systems, CMS can enhance care coordination and improve outcomes for Medicare and Medicaid beneficiaries, while upholding the highest standards of data security and patient trust.

We suggest CMS and ASTP/ONC consider:

- Supporting state-directed investments that align mature standards with Medicaid and public health priorities, ensuring that implementation leverages existing infrastructure and delivers measurable outcomes.
- Mandating all federally regulated healthcare APIs are open and standardized to promote consistency and interoperability.
- Requiring support for bulk data capabilities in APIs to accelerate data exchange and ensure alignment among health IT systems.
- Expanding CMS APIs to include data types that empower providers to improve care quality, efficiency, and patient experience.
- Requiring modern identity solutions for authentication and authorization of patient data.

III. CMS and ASTP/ONC requested feedback on the Trusted Exchange Framework and Common Agreement (TEFCA) regarding necessary changes needed and the evolving role of HIEs within broader data exchange frameworks.

A. TEFCA Barriers and Limitations *PC-10, PC-11, PR-6, PA-1, TD-6, TD-14.

Despite meaningful progress toward nationwide interoperability, health information exchanges (HIEs) continue to encounter challenges in achieving seamless data sharing across the healthcare ecosystem. Persistent issues such as entrenched data silos, inconsistent data standards, and varying technical capabilities among providers continue to limit the full potential of health data for patients, providers, and payers.

The Trusted Exchange Framework and Common Agreement (TEFCA) represents a step forward in establishing a national framework for health information exchange. It lays a foundation for interoperability by enabling point-in-time, document-based exchange across networks. This is an

important advancement, and HIEs are well-positioned to support and bolster TEFCA's implementation by extending its reach and enhancing its impact.

However, as TEFCA continues to evolve, there are inherent limitations to national networks and therefore are opportunities to address critical, state-specific, and community-based data sharing needs that are essential to achieving CMS' goals of improved care coordination, cost reduction, and better outcomes for Medicare and Medicaid beneficiaries. HIEs, with their deep local roots and proven capabilities, can help bridge these gaps.

Key areas where HIEs complement and enhance TEFCA include:

- **Real-Time and Event-Based Exchange**: HIEs offer real-time, event-driven data exchange, such as emergency department alerts and post-discharge notifications, which are vital for timely interventions and care coordination.
- **Support for Diverse Use Cases**: Many HIEs already support public health, behavioral health, and non-medical health related data exchange, which are essential for addressing the needs of critical populations and advancing improved health outcomes.
- **Expanded Use Case Support**: HIEs facilitate HIPAA-permitted uses beyond treatment, including operations and payment, enabling broader support for quality improvement, population health, and value-based care.
- **Proactive Notifications**: HIEs provide push-based alerts like ADT notifications, which are crucial for managing care transitions and reducing avoidable utilization.
- **Community Governance and Trust**: HIEs are often governed by and accountable to local stakeholders, fostering trust and responsiveness to regional needs—especially in rural and underserved areas.
- **Complex Consent Management**: HIEs have experience navigating nuanced and important patient consent requirements, such as state-based requirements and federal requirements such as 42 CFR Part 2, enabling more inclusive and compliant data sharing.

In this way, HIEs serve as a vital complement to TEFCA and other national networks, helping to ensure that the national framework is not only robust but also responsive to the diverse and dynamic needs of communities across the country, and that all data exchange via intermediaries such as HIEs and national networks honor patient data sharing wishes.

B. High-Performing HIE Capabilities in support of TEFCA *PC-10, PC-11, PR-6, PA-1, TD-6, TD-14.

While national frameworks like TEFCA have laid important groundwork for nationwide interoperability, regional HIEs such as Contexture bring critical additional value to the healthcare community. With deep roots in the communities they serve, HIEs offer nimble, locally responsive infrastructure that aligns closely with state priorities and the needs of diverse populations.

Contexture works together with Medicaid programs, public health agencies, and community organizations to deliver a range of impactful services. Initiatives like Electronic Clinical Quality Measures (eCQMs), the Arizona Healthcare Directives Registry (AzHDR), a statewide Crisis Bed Registry and streamlining multiple public health reporting requirements, are examples of how HIEs can address real-world challenges in ways that are tailored, timely, and effective.

- Electronic Clinical Quality Measures (eCQMs) are generated for almost 150 healthcare provider practices to align with alternative payment models in Colorado, to drive quality improvement and improved healthcare outcomes.

- The AzHDR provides individuals with a secure, online way to store and share advance directives with providers, ensuring that care decisions reflect patient preferences and reduce unnecessary interventions. This will significantly impact Medicare.
- The Crisis Bed Registry, created in collaboration with Arizona Medicaid and Solari Crisis and Human Services (the statewide crisis phone line operator) provides real-time visibility into available crisis facilities and beds across the state—streamlining placement and improving crisis response.
- Supporting Arizona and Colorado public health data modernization efforts, through serving as a data intermediary for public health reporting and data exchange, supporting robust and scalable identity management. Supporting states' needs is a key area where Contexture and state and regional HIEs can support data modernization efforts locally and nationally.

These efforts demonstrate how HIEs are already delivering value and helping to advance the goals of CMS and ONC. As TEFCA continues to take shape, there is a valuable opportunity to build on the strengths of both national and regional efforts.

Rather than a one-size-fits-all approach, a layered strategy that integrates TEFCA with the proven capabilities of HIEs can offer the best of both worlds. HIEs like Contexture are well-positioned to serve as data intermediaries, simplifying data access for providers and enhancing the reach of national networks. Many are also ready to connect with Qualified Health Information Networks (QHINs), further strengthening the national exchange infrastructure.

To support this vision, We suggest CMS and ASTP/ONC consider:

- Leverage HIEs as regional API hubs, building on trusted infrastructure to expand access and reduce burden.
- Promote a complementary model that aligns national and regional strengths to accelerate digital health transformation.

By continuing to invest in the capabilities of regional HIEs, CMS can amplify its impact and ensure that interoperability efforts are both scalable and grounded in the realities of care delivery across the country.

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Appendix

Figure 1: Contexture Solutions- Business Lines

Health Information Exchange	Quality Improvement	Data Analytics	Social Determinants of Health	Advance Directives	Public Health
Connecting digital health records to give providers the patient information they need at their fingertips.	Helping healthcare providers meet various federal reporting and compliance requirements.	Providing the information necessary to improve individual and community health.	Connecting healthcare and community service providers to streamline referrals and foster easier access to vital services.	Providing seamless access to advance directives documents across the continuum of care.	Working with public health agencies to improve emergency response, access to vital patient data and reporting.

Figure 2: Contexture's Success – Key HIE Data Metrics

688 DATA SENDERS AZ	16.5K USERS PER MONTH	1105 DATA SENDERS CO
13.2M PATIENTS AZ		14.3M PATIENTS CO
11.7M ALERTS PER MONTH AZ	56+M CCD/HL7 TRANSACTIONS PER MONTH	17.5M ALERTS PER MONTH CO

Figure 3: Contexture's Client Testimonies

Dr. Carroll, CMO of ACO Partners, shared a compelling example of how our Notifications benefit both providers and patients. One patient, who frequently visited the hospital emergency department two to three times a week, was identified through the Contexture notification system and successfully engaged by their ACO team. By directing the patient to a lower-cost urgent care center when appropriate, they significantly reduced the patient's overall need for urgent treatment and associated costs. This proactive approach exemplifies their commitment to providing "the right care in the right place at the right time." Most importantly, patients truly appreciate the improved care and convenience. "In the first two years that ACO Partners connected to and began receiving data from Contexture, the physician-centric organization saw $2.1 million in savings — after its service costs were subtracted — when managing its patient population of approximately 58,000." *-Dr. Andre Carrol, CMO, ACO Partners*

"An unexpected benefit of the Arizona HIE service, Contexture, is the significant improvement in staff scheduling. Arizona Home Care sends nurses and professionals to patients' homes daily. Before receiving Contexture's notifications, staff often arrived at a patient's home only to find the patient at the hospital, resulting in inefficient travel and uncompensated visits. With these notifications, staff can reschedule visits in advance, saving valuable time and resources." *-Jason Smidt, CIO, Arizona Home Care*

Through Contexture Notifications, Horizon Health and Wellness receives notifications when high-needs patients are admitted, discharged, or transferred from the hospital. Dr. Karst admits that he was "shocked to learn how many of our patients were spending so much time in hospitals. We knew we had to do something about it. Contexture has already helped us cut hospital admissions in half for [our] highest users." *-Dr. Fred Karst, COO, Horizon Health and Wellness*

"Hands-down, Contexture has been of monumental importance in helping our organization improve its [electronic clinical quality measure] eCQM results. [Contexture] has provided technical expertise, data and root cause analyses, as well as simple understandable and actionable explanations, to help improve our eCQM results and is indeed a valuable partner for our organization." *-Dr. Savita Ginde, Boulder Valley Health Center.*